ATLANTA HEART & VASCULAR CENTER

2468 Cardiac Drive, Atlanta, GA 30310
Phone: (555) 555-6677 | Fax: (555) 555-6678
NPI: 1357924680

CARDIOLOGY PROGRESS NOTE

SUBMITTED FOR PRIOR AUTHORIZATION REQUEST

Patient: William Charles Thompson
DOB: April 25, 1958 (Age: 65)
MRN: MRN112233
Date of Evaluation: January 20, 2024
Insurance: Humana Inc. (ID: HUM987654321B)
Attending Physician: Dr. Michael Robert Jackson, MD
NPI: 5566778899

SERVICES REQUESTED FOR PRIOR AUTHORIZATION

Prior Authorization Request ID: PA-HUM-CARD-2024-006
Payor: Humana Inc.
Provider: Atlanta Heart & Vascular Center (NPI: 1357924680)
Priority: Urgent

CPT Code	Service Description	Quantity	Unit Price	Total Cost	Service Date
93458	Catheter placement in coronary artery(s) for coronary angiography, including intraprocedural injection(s) for coronary angiography, imaging supervision and interpretation; with left heart catheterization including intraprocedural injection(s) for left	1 procedure	$12,500.00	$12,500.00	01/28/2024

	ventriculography, when performed				
TOTAL REQUESTED AUTHORIZATION				**$12,500.00**	

Place of Service: On Campus-Outpatient Hospital (Code 22)
Procedure Location: Atlanta Heart & Vascular Center

CHIEF COMPLAINT

Chest pain and shortness of breath on exertion, worsening over 3 months.

HISTORY OF PRESENT ILLNESS

Mr. Thompson reports increasing chest pain with exertion over the past 3 months. Pain is substernal, pressure-like, radiates to left arm. Associated with dyspnea and diaphoresis. Symptoms occur with minimal exertion (1 block). No rest pain. Patient describes classic anginal symptoms with progressive worsening, now limiting his daily activities.

PAST MEDICAL HISTORY

Significant for:
• Hypertension - well controlled on medication
• Dyslipidemia - on statin therapy
• Diabetes mellitus type 2 - managed with metformin
• Family history of coronary artery disease (father had MI at age 58)
• No previous cardiac procedures or hospitalizations

PHYSICAL EXAMINATION

Vital Signs:
• Blood Pressure: 152/94 mmHg (elevated)
• Heart Rate: 88 bpm (regular)
• Temperature: 98.4°F
• Respiratory Rate: 18/min
• Oxygen Saturation: 97% on room air

General: Mild distress, appears fatigued

Cardiac: Regular rate and rhythm, no murmurs, gallops, or rubs. Point of maximal impulse not displaced.

Pulmonary: Clear to auscultation bilaterally, no rales or wheezes

Extremities: No edema, peripheral pulses intact

DIAGNOSTIC TESTS

Electrocardiogram (ECG):
• Sinus rhythm
• T wave flattening in leads V4-V6
• Suggestive of anterior wall ischemia

Echocardiogram:
• Ejection Fraction: 45% (mildly reduced)
• Regional wall motion abnormalities in LAD territory
• No significant valvular disease

Stress Test (Exercise Treadmill Test):
• Positive for ischemia at 75% maximum heart rate
• ST depression 2.5mm in leads V4-V6
• Exercise capacity limited by chest pain and dyspnea
• High-risk findings requiring further evaluation

CLINICAL ASSESSMENT AND DIAGNOSIS

Primary Diagnosis:
• I25.10 - Atherosclerotic heart disease of native coronary artery without angina pectoris

Secondary Diagnosis:
• I20.9 - Angina pectoris, unspecified

Clinical Impression:
Mr. Thompson has stable angina with high pre-test probability of significant coronary artery disease. Stress test is positive for ischemia in LAD distribution. Echocardiogram shows regional wall motion abnormalities consistent with coronary artery disease. Given the high-risk findings and progressive symptoms, cardiac catheterization is indicated for definitive diagnosis and potential intervention.

INDICATIONS FOR CARDIAC CATHETERIZATION

1. **Stable angina with high-risk stress test** - Patient has classic anginal symptoms with positive stress test
2. **Positive stress test with significant ST depression >2mm** - Indicates high-grade coronary stenosis
3. **Regional wall motion abnormalities on echocardiogram** - Consistent with prior ischemic injury
4. **Symptoms limiting daily activities** - Progressive functional limitation
5. **High pre-test probability of CAD** - Based on age, gender, and multiple risk factors
6. **Failed optimal medical therapy** - Symptoms persist despite appropriate medications
7. **Need for risk stratification** - To determine appropriate treatment strategy

PROPOSED PROCEDURE PLAN

Procedure: Diagnostic cardiac catheterization with coronary angiography and left heart catheterization

Approach: Right radial artery access (preferred) or right femoral artery

Components:
• Selective coronary angiography of left and right coronary systems
• Left ventriculography to assess wall motion and ejection fraction
• Hemodynamic assessment
• Potential for ad hoc percutaneous coronary intervention if appropriate

Scheduled Date: January 28, 2024
Location: Atlanta Heart & Vascular Center Cardiac Catheterization Laboratory

MEDICAL NECESSITY JUSTIFICATION

Cardiac catheterization is medically necessary for Mr. Thompson due to his high-risk presentation with stable angina, positive stress test with significant ST changes, and regional wall motion abnormalities on echocardiogram. The procedure is indicated per ACC/AHA guidelines for patients with high-risk non-invasive testing results. Without definitive coronary anatomy assessment, the patient remains at risk for major adverse cardiac events including myocardial infarction and sudden cardiac death.

RISKS AND BENEFITS DISCUSSION

Patient has been counseled on the risks and benefits of cardiac catheterization including but not limited to: bleeding, infection, contrast nephropathy, stroke, myocardial infarction, and death (risk <0.1%). Benefits include definitive diagnosis of coronary anatomy, risk stratification, and potential for therapeutic intervention. Patient understands and agrees to proceed with the procedure.

Attending Cardiologist:

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Dr. Michael Robert Jackson, MD
Board Certified Cardiologist
License #: MD67890GA
NPI: 5566778899
Date: January 20, 2024